

Select RR-TB medications according to chosen RR-TB regimen

Regimen	Intensive phase	Continuation phase
Shorter regimen	- Bedaquiline (at least 6 months) - Linezolid (2 months only) - Levofloxacin - Clofazimine	- High dose isoniazid¹ - Pyrazinamide - Ethambutol
Longer regimen This longer regimen is for uncomplicated cases as chosen in step 2 ➔ 102. Avoid and discuss instead if any of: - Hb < 8 - CNS disease (TB meningitis or brain tuberculoma) - Resistance to FLQ, BDQ, LZD or CFZ - Patient is a TB contact² of index patient with resistance to FLQ, BDQ, LZD or CFZ or failing treatment	- Bedaquiline - Linezolid - Levofloxacin	- Clofazimine - Terizidone

Note: manage the patient with RR-TB at a health facility that has reliable access to RR-TB medications and monitoring equipment available.

Dose RR-TB medications according to weight

Medication	Daily dose				Note
	30-35kg	36-45kg	46-70kg	> 70kg	
Bedaquiline (BDQ)	400mg daily for first 2 weeks - Then 200mg 3 days a week (Mon/Wed/Fri)				If previous cardiac ventricular arrhythmias, severe coronary artery disease, known or family history of prolonged QT syndrome, previous intolerance to bedaquiline, or on other QT-prolonging medications (anti-arrhythmics, tricyclic antidepressants and antipsychotics), discuss with PCAC/NCAC.
Linezolid (LZD)	600mg	600mg	600mg	600mg	Avoid starting if Hb < 8, neutrophils < 0.75 or platelets < 50: discuss instead with PCAC/NCAC.
Levofloxacin (LFX)	750mg	750mg	1000mg	1000mg	
Clofazimine (CFZ)	100mg	100mg	100mg	100mg	If on other QT-prolonging medications (anti-arrhythmics, tricyclic antidepressants and antipsychotics), discuss with PCAC/NCAC.
Isoniazid	High dose (hdINH)	450mg	450mg	600mg	If phenotypic DST confirms sensitivity to INH, reduce to normal dose INH.
	Normal dose (INH)	200mg	300mg	300mg	
Pyrazinamide (Z)	1000mg	1500mg	1500mg	2000mg	
Ethambutol (E)	800mg	800mg	1200mg	1200mg	
Terizidone (TRD)	500mg	750mg	750mg	750mg	If previous psychosis, avoid terizidone and present to PCAC/NCAC ³ .
Delamanid (DLM)	100mg 12 hourly	100mg 12 hourly	100mg 12 hourly	100mg 12 hourly	
PAS	8g	8g	8g	8g	
Ethionamide (ETO)	500mg	500mg	750mg	750mg	
Moxifloxacin (MFx)	400mg	400mg	400mg	400mg	If on other QT-prolonging medications (anti-arrhythmics, tricyclic antidepressants and antipsychotics), discuss with PCAC/NCAC.
Amikacin (Am) (15-20mg/kg)	625mg	750mg	750-1000mg	1000mg	Ensure audiometry (hearing test) done at baseline and then monthly.
Rifabutin	300mg	300mg	300mg	300mg	- Give for 6 months if heteroresistance confirmed by laboratory and approved by PCAC/NCAC. - If on lopinavir or atazanavir, reduce rifabutin dose to 150mg daily.

BDQ – bedaquiline; **CFZ** – clofazimine; **FLQ** – fluoroquinolone (e.g levofloxacin or moxifloxacin); **LZD** – linezolid;

¹If phenotypic DST confirms sensitivity to INH, reduce to normal dose INH. ²A TB contact refers to a patient who shared an enclosed space (at work, socially, in a hostel, or in a household setting), for ≥ 1 night or for frequent/extended daytime periods, with an adult/adolescent with pulmonary TB ("index patient"), during the 3-month period before the index patient started their TB treatment. ³Start other medications while awaiting response from PCAC/NCAC.